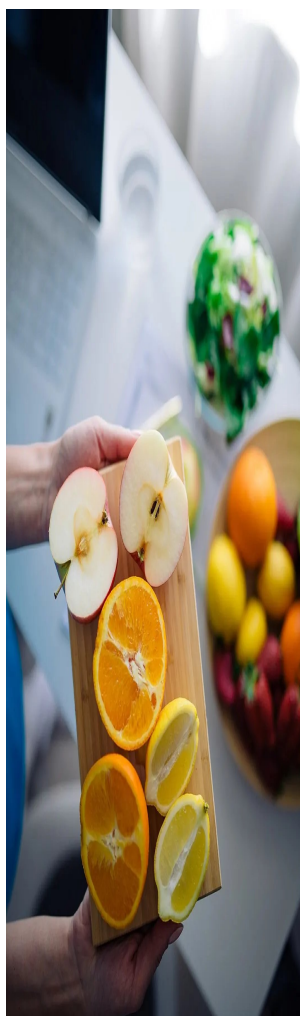


What Sustained Remission Means in RA

Remission isn't cure. Understanding the difference changes how you plan.

By **Living With Arthritis Editorial Team** · Reviewed by **Clinical Review Board** (Multidisciplinary UK clinicians) · 2026-06-16 · livingwitharthritis.org.uk

Overview



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Remission isn't cure. Understanding the difference changes how you plan.

What the evidence shows

UK guidance from NICE, the British Society for Rheumatology and Arthritis UK consistently emphasises a combined approach: appropriate medication, regular movement, and supportive lifestyle changes. Recent reviews continue to back this consensus.

What this means for you



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- ****Talk to your clinician.**** Personalised advice always beats blanket rules.
- ****Track your symptoms.**** Use a simple diary so you can spot patterns over weeks, not days.
- ****Move within comfort.**** Gentle, regular activity protects joints and mood.

- ****Watch the warning signs.**** New swelling, fever, or rapid loss of function deserve same-week review.
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Practical next steps

1. Read our companion piece for a deeper dive: [\[related guidance\]](#)[\(/conditions/rheumatoid-arthritis\)](#).
 2. Bring three questions to your next GP or rheumatology appointment.
 3. Share this article with a family member who supports you day to day.
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Editorial standards



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This article was prepared by the Living With Arthritis Editorial Team and reviewed by our Clinical Review Board against UK NICE guidance and peer-reviewed sources. We update content at least annually.

What the Evidence Says

The consensus among rheumatology experts and patient advocacy groups is that achieving remission in rheumatoid arthritis (RA) is a primary treatment goal. Systematic reviews of long-term studies consistently show that individuals who reach and maintain remission experience better quality of life, less joint damage, and improved physical function compared to those with ongoing disease activity. This evidence underpins current clinical guidelines, which strongly advocate for a 'treat-to-target' approach, meaning adjusting treatments until remission or very low disease activity is achieved.

However, clinical guidance also highlights that "remission" can be defined in different ways, from patient-reported outcomes to strict laboratory and physical examination criteria. Randomised trials examining various drug therapies often use these stricter definitions, showing that while remission is increasingly achievable, particularly with early and aggressive treatment, it often requires ongoing medication rather than being a permanent 'cure'.

Therefore, the evidence points to remission as a stable state of minimal or no disease activity, usually maintained through careful management, rather than a total eradication of the disease. This distinction is crucial for managing expectations and planning long-term care.

Common Mistakes to Avoid

* **Stopping medication when feeling well:** Your treatment is likely what's keeping you well; stopping without medical advice can cause flare-ups. *Always discuss medication changes with your doctor.* * **Ignoring subtle symptoms:** A slight stiffness or an odd ache could be an early warning sign, not just a 'bad day'. *Keep your symptom diary updated and discuss any new or worsening symptoms.* * **Overdoing it on good days:** Pushing too hard when you feel good can lead to a setback and increased fatigue. *Pace yourself and listen to your body, even in remission.* * **Neglecting other health aspects:** Diet, sleep, and stress management play a huge role in overall well-being with RA. *Focus on a holistic approach to your health, not just medication.* * **Expecting a 'cure' instead of management:** While remission is fantastic, it's usually ongoing management rather than a permanent fix. *Understand that sustained remission often means continued vigilance and treatment.*

When to Speak to a Clinician

Even in sustained remission, it's important to be aware of signs that might indicate a change in your condition or require medical review. Persistent symptoms should always be checked out.

* **New, persistent joint pain or swelling:** Especially if it lasts for more than a few days, or affects joints previously unaffected. * **Unexplained fever or generally feeling unwell (malaise):** These can sometimes indicate a flare or an infection. * **Significant increase in fatigue:** If your energy levels suddenly plummet and rest doesn't help. * **Rapid loss of function in a joint:** If you find you suddenly can't move a joint as you usually could. * **Side effects from medication:** Any new or worsening side effects from your prescribed drugs should always be discussed.

Frequently Asked Questions

Can I reduce my medication once I'm in sustained remission?

Sometimes, yes, but this must be done very carefully and under the guidance of your rheumatologist. Reducing medication needs to be a shared decision, considering your individual circumstances and risk of flare.

How is 'sustained remission' actually measured? It's measured using a combination of factors, including your own report of symptoms (like how much pain or stiffness you have), your doctor's examination of your joints, and blood tests to check for inflammation markers. This data is often fed into a specific scoring system.

Does being in remission mean I can stop exercising? No, regular exercise remains very important. It helps maintain joint flexibility, muscle strength, and overall well-being, even when you're feeling good. Your specialist or a physiotherapist can advise on suitable activities.

Q&A

Question: What is the key thing to know about what sustained remission means in ra?

Short answer: Sustained remission in rheumatoid arthritis means stable minimal or no disease activity, not a cure. The article says it is a key treatment goal, often maintained through ongoing medication, monitoring, gentle regular movement and lifestyle support. It advises personalised clinician guidance, symptom tracking, pacing activity, and prompt review for swelling, fever, rapid function loss or medication side effects.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis · livingwitharthritis.org.uk